

Preceptor Accountability Planning Guide

An operational framework for clear teaching expectations, feedback, and follow-through

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For surgical APP programs and preceptor teams. Adapt the prompts through your normal clinical education, APP leadership, HR, legal, and governance review.

Why a named preceptor is not yet a teaching system

COMMON STARTING POINT	PRACTICAL RISK
A surgeon or APP is assigned to teach without preparation	Teaching expectations vary by person and service line.
The role is described only as "teach the APP"	The learner may not know what to observe, practice, discuss, or document.
Feedback is informal or delayed	Questions, uncertainty, and improvement needs may stay hidden.
Teaching is added to clinical duties without review	Preceptorship competes with cases, coverage, patient care, and other work.
Recognition and escalation are undefined	Strong teaching is invisible, and role fit is discussed only after frustration.

Use this guide as a planning aid

The framework is not a competency standard, certification pathway, staffing rule, compensation schedule, safety metric, or employment process. It offers questions and examples for a locally approved approach.

1) The preceptor competency model

Define observable teaching practices before deciding how to review them.

PRACTICE	WHAT IT LOOKS LIKE	LOCAL REVIEW PROMPT
Structured demonstration	Breaks a procedure or workflow into teachable steps and explains what matters.	Where does the learner need a demonstration, narration, or task map?
Progressive responsibility	Increases responsibility based on observed performance, supervision, local policy, and team judgment - not a calendar alone.	What evidence and supervision decisions inform the next step?
Specific feedback	Names the behavior, why it matters, and what to try next.	Can the learner leave the conversation knowing the next practice step?
Structured debriefing	Reviews what happened, what was learned, and what to focus on next.	What debrief timing and format fit the service line?
Safe learning environment	Makes it possible to ask questions, name uncertainty, and raise concerns through the appropriate route.	How will the program protect questions and concerns from being dismissed?

Example language

"Let us name the task, the supervision available today, what you will do, and when we will debrief." Adapt the language to the role, procedure, policy, and team involved.

2) Preceptor development stages

Use stages as local development language, not as a universal credential or certification.

STAGE	LOCAL DESCRIPTION	QUESTIONS TO REVIEW
Preparing	Role expectations and teaching basics are not yet complete.	What preparation is required before independent teaching?
Supported	The preceptor has completed local preparation and has access to coaching or a mentor.	What support and observation are available?
Established	The preceptor uses the agreed teaching, feedback, and review practices consistently.	What evidence shows the process is working for learners and the team?

STAGE	LOCAL DESCRIPTION	QUESTIONS TO REVIEW
Contributing	The preceptor helps improve the program and supports other preceptors.	What leadership, education, or improvement role is appropriate?

3) Preceptor preparation curriculum

A modular outline for local education planning. Duration and sequence are examples, not requirements.

MODULE	EXAMPLE FOCUS	PRACTICAL OUTPUT
1. How people learn clinical work	Deliberate practice, cognitive load, observation, and error-based learning.	A discussion of how learners will practice and receive support.
2. Demonstration and task breakdown	Narration, task decomposition, and graduated responsibility.	A task map for common service-line or procedural work.
3. Feedback that changes behavior	Timely, specific feedback and managing difficult conversations.	Practice feedback on representative scenarios.
4. Debriefing and case conference	A short structure for what happened, what it means, and what comes next.	A debrief format that fits the team's workflow.
5. Supervision and responsibility decisions	Observed performance, case or task complexity, local policy, and escalation.	Local criteria and decision owners for changing responsibility.
6. Documentation and review	Honest, specific records that support learning and appropriate follow-up.	A reviewed example of useful documentation.

Before adoption

Confirm the curriculum with the people responsible for clinical education, APP leadership, supervision, credentialing, privileging, HR, legal review, and safety reporting. This guide does not decide those requirements.

Suggested planning rhythm

Choose a cadence that fits the program rather than treating this schedule as a standard.

Start with role expectations and teaching basics. Observe or discuss a small number of teaching encounters. Review learner and preceptor feedback. Agree on one improvement action, owner, and review date. Repeat through the program's normal education and governance cycle.

4) Feedback and preceptor review

Use a small set of prompts to make teaching behavior visible without turning the guide into a personnel scorecard.

Possible learner feedback prompts - adapt wording, timing, anonymity, and response handling to local policy:

- The preceptor explained the task or workflow clearly enough for me to understand the next step.
- The feedback named a behavior and gave me a practical way to improve.
- The preceptor adjusted responsibility with appropriate supervision and explanation.
- I could ask questions or name uncertainty through a safe and appropriate route.
- Expectations, follow-up, and review timing were clear.
- The preceptor completed required documentation or feedback on the agreed schedule.

Do not overread a score

A feedback response is one input. Interpret it with context, response rate, role, service line, learner experience, supervision, and the program's normal review process. Do not use this guide to make individual clinical competence, employment, credentialing, privileging, or disciplinary decisions.

5) Choose locally reviewed signals

Use signals to start a conversation, not to create universal green, yellow, or red thresholds.

SIGNAL	WHAT IT MAY HELP YOU NOTICE	CHECK BEFORE USE
Feedback completion	Whether agreed learner or preceptor feedback is happening.	Who receives it, when, and how privacy is protected?
Debrief and follow-up cadence	Whether planned conversations occur and produce next steps.	What counts as a useful debrief in this setting?
Responsibility decisions	Whether supervision and progression decisions are explainable and documented.	Which policy, role, and clinical governance requirements apply?
Concerns raised and reviewed	Whether questions, errors, or safety concerns reach the appropriate process.	What is the approved reporting and escalation route?
Learner and preceptor experience	Where expectations, workload, support, or role fit need attention.	What context prevents an isolated response from being overread?

6) From review to action

Data has value only when the team knows what it will discuss, change, and revisit.

STEP	WORKING QUESTION	OUTPUT
Review	What did we hear or observe, and whose perspective is missing?	A short, contextual summary.
Interpret	What may explain the pattern? What should not be inferred?	A bounded interpretation and open questions.
Act	What one change is safe, feasible, and within the team's authority?	Owner, support, and next action.

STEP	WORKING QUESTION	OUTPUT
Escalate	Does this belong with clinical education, APP leadership, HR, legal, safety, credentialing, or privileging?	A documented handoff through the right route.
Revisit	When will we see whether the action helped or needs adjustment?	Review date and learning point.

7) When a preceptor should step back

Role reassignment can be a professional decision when teaching fit, support, or program needs do not align.

A step-back conversation may be appropriate when documented support has not resolved a recurring concern, the preceptor identifies that teaching is not a good fit, workload or role design makes the assignment unrealistic, or the program needs a different supervision arrangement. Use the organization's approved process. Do not treat a sample score or threshold in this guide as an automatic trigger.

- Name the concern and the evidence without turning it into a character judgment.
- Offer appropriate support, coaching, and a reasonable review period where applicable.
- Clarify who decides about the role and what policy or governance process applies.
- Protect the learner, patients, team, and preceptor through the appropriate escalation route.

Important boundary

This guide does not establish a disciplinary pathway, employment consequence, clinical supervision rule, credentialing requirement, privileging decision, or safety investigation.

8) Recognition and program self-assessment

Make teaching visible while keeping compensation and career decisions inside local policy and review.

RECOGNITION OPTION	LOCAL ADAPTATION PROMPT
Thank-you or department recognition	What recognition is meaningful and equitable for this team?
Teaching portfolio or education contribution	What work can be documented for the appropriate review process?
Protected preparation or debrief time	What schedule or workload change would make teaching feasible?
Mentoring or curriculum contribution	Who is ready to support other preceptors or improve the program?
Compensation or promotion consideration	What policy, budget, HR, and leadership review is required?

Use the following seven dimensions as a discussion checklist. The score is a local conversation starter, not a validated maturity model or a comparative ranking.

1. Preceptors receive preparation before being assigned an APP. Discussion note:

2. Teaching expectations are defined with observable behaviors. Discussion note: _____
3. Learner and preceptor feedback is collected through an appropriate process. Discussion note: _____
4. Review findings are discussed with the people who can act on them. Discussion note: _____
5. Preceptors have a supported way to change or step back from the role. Discussion note: _____
6. Teaching work is recognized through an equitable, locally reviewed approach. Discussion note: _____
7. The program examines patterns without attributing outcomes to one person without context. Discussion note: _____

Next action

Choose one dimension. Name the owner, the people who need to review it, the first practical change, and a date to revisit the result. Keep patient, employee, and other sensitive information out of this planning document.

A one-page conversation plan

Use this page in a leadership or preceptor-program discussion.

PROMPT	NOTES
The preceptor-program issue we want to address	
What we observe or hear - without adding patient or employee identifiers	
What we should not infer from the information	
The first bounded action	
Owner and reviewers	
Support or policy check needed	
Review date and learning question	

Keep the scope clear

This free guide helps a program examine teaching infrastructure. It does not assess an individual clinician, certify a preceptor, replace education or HR processes, provide legal or clinical advice, or guarantee APP retention, performance, safety, or other outcomes.

Use with the APP leaders, preceptors, clinical educators, and governance partners who own the work. Adapt every example to the organization's policies, staffing model, service line, supervision structure, and review requirements.

Before local adoption

A final review checklist for the people accountable for the program.

- Clinical education and APP leadership reviewed the teaching and supervision examples.
- HR and legal reviewers confirmed that recognition, compensation, documentation, and role-change language fits local policy.
- Safety, escalation, credentialing, and privileging owners confirmed the appropriate boundaries and reporting routes.
- The team identified what information may be collected, who may see it, and how it will be protected.
- The program agreed which items are examples, which are local requirements, and which remain open decisions.
- The owner and review date for the first improvement action are recorded.

Free resource boundary

This guide is complete as a planning aid. A separate implementation toolkit may provide editable files for role expectations, responsibility mapping, check-ins, action tracking, and review meetings, but neither resource substitutes for local clinical, HR, legal, credentialing, privileging, or governance decisions.

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